

Stream — Field One-Pagers

Independent Pediatrics 19 Aug 2026 For the GTM team — not for distribution to prospects

FIELD SHEET

Independent Pediatrics

The deepest thing we've built and the least proven. Sell the fax pile today; sell the well-visit panel as what it grows into.

WHO TO CALL, WHO TO SKIP

CALL

- + **3–15 clinicians** — three is the floor, four or more is the target
- + Owner-physician or practice manager — they own both the schedule and the money
- + No EHR, a cheap one, or one whose recall report nobody trusts
- + Already bought a scribe (they've cleared the AI objection and found it didn't fix the paperwork)

SKIP

- Hospital-owned or system-affiliated
- 25+ clinicians, or anyone with an IT director
- **Under 3 clinicians** — self-serve trial, not a booked demo
- Anyone who makes EHR write-back a condition of the trial

THE OPENER

When an ED discharge summary comes back for one of your patients, who reads it — and where does it end up? Every practice has a bad answer: scanned to a folder, or sitting in a pile. You're asking about a workflow, not claiming a capability — that's why it gets replies. Subject lines to test: *the discharge summary pile · who reads your faxes*. Never "AI scribe," never "save 2 hours a day."

THE THREE BEATS

- 1 Walk in ready.** Before you open the door: every active problem, everything the outside world sent, and what's waiting on you — one screen.
- 2 Leave with the note done.** Records the visit, writes it up problem by problem in your voice, suggests the codes. One click to paste wherever it goes.
- 3 Nothing falls off.** Faxes and discharge summaries read themselves and file under the right problem. Well visits surface on the AAP schedule as your panel comes through.

THE DEMO — 20 SECONDS, RUN IT ON EVERY CALL

Upload a real ED discharge summary. Watch Stream pull out the clinical content and file it under the right problems. Click **View source** on the surprising one and

land on the highlighted sentence in the original fax. Then stop talking.

Second demo, *only* if `task_rules` is on for that account: filter the panel to 1–5y, sort by days overdue, export CSV. "That's your call list."

THE NUMBER — MAKE THEM BUILD IT

Named references yes, time-saved statistic no — don't invent one. Ask: *what do you collect on a well visit, and how many kids are behind?* Multiply it in front of them; their number beats ours.

Say this before they discover it.

The engine only counts visits it has seen, and won't accuse a family of missing one that happened before Stream arrived. The panel fills in as patients come through — useful in months, complete after a full visit cycle. **The document workflow works the first week.** Lead with that and let the panel compound; never promise a populated call list on day one.

OBJECTIONS

"We already have an EHR."

Good — keep it. Stream works alongside it. Nothing to install, no interface, no IT project, no implementation fee.

"Our EHR already does recall."

Pull it up while we're on the call. Does it know the difference between a visit that's clinically due and one the payer will cover yet? And does it reach the clinician at the visit, or land in a report nobody opens?

"So we type everything twice?"

One click, formatted for any field — rich text and plain text at once, or section by section. It's a paste, not a retype. *Two sentences, then move on; arguing a fair objection only makes it bigger.*

"Does it make things up?"

Every claim links back to the exact sentence it came from, and anything unverifiable is suppressed. Outside records never silently become the clinician's words. Show it — don't describe it.

PRICING

Pro \$149/clinician/mo, or **\$99 annually** · staff seats **\$25** (first free) · 30-day trial, no card · uploads included · fax number **free for 30 days**.

APPLIES TO EVERY SEGMENT

Guardrails

Everything here was verified against the product, not the roadmap. A claim from the left column is the fastest way to lose a deal in diligence.

Booking floor — 3 clinicians minimum, 4 or more preferred.

No demo with a practice under three clinicians. At \$149 a seat, a one- or two-person practice doesn't cover the cost of sourcing the meeting — that demand is real, but it arrives through the self-serve trial, not a booked call.

Confirm headcount before the invite goes out. Count physicians, NPs and PAs; not front-desk, billing or MAs — those are \$25 seats.

NEVER SAY / SAY INSTEAD

NEVER SAY	SAY INSTEAD
✗ Integrates with your EHR · syncs with · connects to · writes back	Works alongside any EHR. No integration, no IT project, no implementation fee. (<i>The old "native integrations coming" claim was removed from the website in Aug 2026 — it was never true. Don't reintroduce it on a call.</i>)
✗ SOC 2 compliant / certified	BAA included in our terms. Per-tenant data isolation, role-based access control, audit logging, two-factor authentication.
✗ HIPAA certified	HIPAA-aligned architecture. (No vendor can be HIPAA certified — the phrase reads as amateur to anyone who'd know.)
✗ Your overdue list is ready on day one	The document workflow works week one. The panel view fills in as your patients come through.
✗ Any mention of Form Filler, automatic chart extraction into structured tables, or style-learning from a sample note	Nothing. Not shipped. Don't hint at it either.
✗ Unlimited history on every plan	Lite / Quick Encounter deletes after 48 hours — state it plainly in all Lite material or it's a refund and a support fire. Lite (\$59, scribe only, no chart) isn't publicly sold: never put a practice on it.
✗ Clinicians save X minutes a day	We have no time-saved statistic — don't invent one. We <i>do</i> have three named physician references and 30,000+ documented encounters. Use those, and build the ROI number out of the prospect's own figures.

BEFORE ANY DEMO

Confirm flag state on the account — these are per-tenant and may be off:

task_rules document_excerpts tasks_module

outbound_faxes journal_club help_chatbot

merge_inline_recommendation

Documents still carries a **Beta** badge. One-click demo charts exist for family medicine, pediatrics, cardiology, endocrinology and behavioral health — ask Jacob for access, and **demo a pediatrician a pediatric panel.**

SET THESE EXPECTATIONS IN ONBOARDING, NOT IN THE WILD

- Fax needs a Spruce account or a Stream-provisioned number: ~15 minutes of admin *plus* a manual step on our side. Not self-serve — don't promise same-day fax.

- Excerpt review is one-way: Accept and No-action are final (Re-tag is the exception).
- Regenerating a note replaces manual edits. Teach generate-once-then-edit-in-place — the most common way a new user loses work.

WHAT WE STILL OWE THE FIELD

- **Proof.** Three named physician references and 30,000+ documented encounters exist and are on the website — use them. Still missing: retention figures and a defensible time-saved number, so ROI is still built from the prospect's own numbers.
- **A security page.** Small-practice buyers ask, and the Trust Center link is currently commented out of the app.
- **Prior-well-visit import.** The single build that would turn the pediatrics sheet from a promise into a demo.

Derived from the engineering handoff of 19 Aug 2026 and a direct read of the source. Claims about what the product does were checked in code; claims about who wants it are hypotheses with one design partner behind them. Update this sheet when that changes.